



Medicare & Mental Health Coverage



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What's Changed?

- Added more information about digital mental health treatment (DMHT) devices (page 5)
- Updated the description of HCPCS code G0136 (page 36)

Substantive content changes are in dark red.

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Medicare-covered behavioral health services, typically known as mental health and substance use services, can affect a patient's overall well-being. It's important to understand Medicare's covered services and who can provide them.

Anyone experiencing a mental health crisis, including a substance use crisis or thoughts of suicide, can get confidential support 24/7 by calling 988 or visiting 988lifeline.org. Visit the [SAMHSA 988 Partner Toolkit](#) for information and resources.

Medicare-Covered Mental Health Services

We **may cover** these behavioral health and wellness services:

- [Alcohol misuse screening and counseling](#) for adults who use alcohol but aren't dependent. If you detect misuse, we cover up to 4 brief, face-to-face counseling sessions per year if the patient is competent and alert during counseling.
- [Alcohol treatment](#), behavior modification using [chemical aversion therapy](#), detoxification, outpatient hospital treatment, and rehabilitative services, including [inpatient hospital stays](#).
- [Annual wellness visit](#) (AWV) to develop or update a personalized prevention plan, including a physical activity and nutrition risk assessment and a depression screening.
 - If you detect cognitive impairment at an AWV or other routine visit, you may perform a more detailed [cognitive assessment](#) and develop a care plan during a separate visit
 - The AWV can be a community health integration (CHI) or principal illness navigation (PIN) initiating visit
- [Advance care planning](#) to discuss a patient's health care wishes, including an advance directive, if they can't make decisions about their care. It can be part of the AWV or a separate Medicare Part B service.
- [Behavioral health integration](#) (BHI) by clinical staff to assess, monitor, and plan care, as well as psychiatric collaborative care services, a specific type of BHI that uses a psychiatric consultant to enhance primary care services.
- Caregiver training services when a physician, non-physician practitioner (NPP), or therapist provides them as part of the patient's individualized treatment plan or therapy plan of care. You can provide these through telehealth.
- Caregiver-focused behavioral health risk assessment of their own behavior and health risks, which benefits the patient.

- CHI services to help patients identify and connect with appropriate clinical and social support resources when they have upstream drivers that affect the diagnosis and treatment of their medical problems.
 - Practitioners may provide CHI services monthly, as medically necessary, following an initiating Evaluation and Management (E/M) visit (CHI initiating visit) when the practitioner identifies upstream drivers that significantly limit their ability to diagnose or treat the patient problems addressed in the visit.
 - Community-based organizations may employ community health workers, care navigators, peer support specialists, and other auxiliary personnel if the billing practitioner provides the required supervision.
- [Chronic care management](#) (CCM) and complex CCM for patients with multiple chronic conditions placing them at high risk of death, acute exacerbation or decompensation, or functional decline, including chronic pain management for patients with chronic pain.
- [Cognitive assessment and care planning](#) (CPT code 99483), a comprehensive evaluation of a new or existing patient who exhibits cognitive impairment signs and symptoms, required to establish or confirm a diagnosis, an etiology, and a condition's severity.
- [Depression screening](#), up to 15 minutes annually, when staff-assisted depression care supports can ensure accurate diagnosis, effective treatment, and follow-up. At a minimum, staff-assisted support is clinical staff in a primary care setting who can advise the physician of screening results and coordinate treatment referrals. You can provide this through telehealth.
- [Digital mental health treatment](#) (DMHT) devices provided "incident to" professional behavioral health services used along with ongoing treatment under a behavioral health treatment plan of care.
 - The billing practitioner must prescribe or order the DMHT device that FDA has cleared under section 510(k) of the [Food, Drug, and Cosmetics Act](#) or granted De Novo authorization, and in each case classified at [21 CFR 882.5801](#) or [21 CFR 882.5803](#). The billing practitioner also must incur the cost of providing the DMHT device to the patient as a supply.
 - A [physician or other practitioner](#) who's authorized to diagnose, evaluate and treat a mental health disorder may prescribe or order a DMHT device, as permitted under the device's FDA clearance in accordance with state prescriptive authority and may report HCPCS code G0552. The patient can use the DMHT device at home or in an outpatient setting, if that's how FDA classified the device.
 - **The patient must have a [mental health condition diagnosis](#), but the billing practitioner doesn't need to be the practitioner who made the diagnosis.**
- Electroconvulsive therapy (ECT), which involves passing small electric currents through the brain, intentionally triggering a seizure, to treat major depressive disorder and other clinical conditions.
- Health and behavioral assessment and intervention that identifies or treats psychological, behavioral, emotional, cognitive, and social factors important to preventing, treating, or managing physical health issues.

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- Hypnotherapy.
- [Initial preventive physical exam](#) to review medical and social health history and provide preventive services education, counseling, and referral, as appropriate.
- Intensive outpatient program (IOP) services.
- Medication for opioid use disorder (MOUD) management when a patient agrees to a medication trial period and its effectiveness is monitored.
- [Medications for substance use disorders](#) (SUDs), which includes medications for alcohol use disorder (MAUDs) and MOUDs when you use them with counseling and behavioral therapy to treat SUDs. We also pay for certain medications and services a certified opioid treatment program (OTP) provider uses to treat opioid use disorders (OUDs), including MOUD in the emergency department (ED) (using HCPCS code G2213).
- [Narcosynthesis](#), used only by a medical doctor (MD) or doctor of osteopathy (DO) for administering sedative or tranquilizer drugs, usually intravenously, to relax the patient and remove inhibitions for discussion of subjects difficult for the patient to discuss freely in the fully conscious state.
- [OTPs](#), which provide counseling and MOUDs—including methadone, buprenorphine, naltrexone, and nalmefene—for patients diagnosed with OUD. OTPs must be SAMHSA-certified and accredited by an independent, SAMHSA-approved accrediting body.
- [Partial hospitalization program](#) (PHP), a structured, intensive, outpatient psychiatric services program that's an alternative to inpatient psychiatric care, with care provided during the day (not requiring an overnight stay) through a hospital outpatient department or community mental health center (CMHC).
- Physical activity and nutrition risk assessment (using HCPCS code G0136). You may provide this assessment with an E/M visit, AWV, or behavioral health office visit, such as a psychiatric diagnostic evaluation and an HBAI.
- [PIN services](#), which help patients diagnosed with high-risk conditions (for example, mental health conditions, SUD, or cancer) identify and connect with appropriate clinical and social support resources. You can perform these after a psychiatric diagnostic evaluation or a health behavior assessment and intervention (HBAI), which can serve as the initiating visit.
- PIN-peer support services, which are like PIN services but have more focus on services performed by peer support specialists under general supervision.
- Psychiatric evaluation to create a treatment plan based on a psychiatric disorder's causes, symptoms, course, and consequences.
- Psychoanalysis that treats mental disorders, in which people trained as psychoanalysts investigate the interaction of conscious and unconscious elements.

- Psychological and neuropsychological testing.
- Psychotherapy, including:
 - Family psychotherapy with or without the patient present, as medically reasonable and necessary, with patient treatment as the primary purpose
 - Individual and group psychotherapy—individual therapy with 1 or more therapists or more than 1 person in a therapy session with 1 or more therapists
 - Interactive psychotherapy
 - [Psychotherapy for crisis](#)
- Safety planning intervention (SPI) and post-discharge phone follow-up contacts intervention (FCI).
- [Screening, brief intervention, and referral to treatment](#) (SBIRT) services, which are early interventions and treatment for people with SUDs, as well as those at risk of developing these disorders.
- SUD treatment, including treatment for OUD, in:
 - An [office setting](#) for OUD management, counseling, and services (using HCPCS billing codes G2086, G2087, and G2088). Treatment includes overall management, care coordination, and individual and group psychotherapy.
 - A patient's home (which is an acceptable telehealth originating site for treating substance use or a co-occurring mental health disorder).
- Telehealth—defined as 2-way, interactive, audio-video technology—to diagnose, evaluate, or treat certain mental health or SUDs if the patient is in their home. Practitioners must be able to provide 2-way, real-time, audio-video technology services but may use audio-only technology given an individual patient's technological limitations, abilities, or preferences. We cover telehealth for behavioral and mental health on a permanent basis.
- Therapeutic activities that can improve the patient's condition, like occupational therapy, recreational therapy, and milieu therapy.
- [Tobacco use cessation counseling](#), 2 cessation attempts per year (each attempt may include a maximum of 4 intermediate or intensive sessions, with the patient getting up to 8 sessions per year). You can provide these through telehealth.
- Transcranial magnetic stimulation for treating severe major depressive disorder.
- [Transitional care management](#) within 30 days of inpatient hospital setting discharges, interactive contacts, certain non-face-to-face services, and face-to-face visits.

The [Roadmap to Behavioral Health](#) helps patients understand how to use their health coverage to improve their mental and physical health.

Non-Covered Mental Health Services

We don't cover these mental health services:

- 24-hour in-home care
- Adult day health programs, like structured therapeutic health services and supervised activities
- Biofeedback training
- Environmental intervention or modifications
- Experimental treatments, including hemodialysis specifically for treating schizophrenia
- Interpreting or explaining results or data
- Massage therapy
- Outpatient meals as part of a PHP
- Pastoral counseling
- Preparing reports
- Transportation to appointments

Same-Day Billing for Mental Health and Primary Care Services

Integrating mental health and SUD services addresses all patients' needs, whether they get care in a traditional primary care setting or a specialty mental or SUD health care setting. Services include:

- Primary health care services
- Mental health care services (including substance use treatment)
- Alcohol and substance use (other than tobacco use) structured assessment and intervention services (SBIRT services) billed under 1 of these HCPCS codes:

G2011	Alcohol and/or substance (other than tobacco) misuse structured assessment (e.g., AUDIT, DAST), and brief intervention, 5–14 minutes	
G0396	Alcohol and/or substance (other than tobacco) misuse structured assessment (e.g., AUDIT, DAST), and brief intervention 15 to 30 minutes	
G0397	Alcohol and/or substance (other than tobacco) misuse structured assessment (e.g., AUDIT, DAST), and intervention, greater than 30 minutes	

AUDIT: Alcohol Use Disorders Identification Test

DAST: Drug Abuse Screen Test

Part B pays for reasonable and necessary integrated health care services provided on the same day, to the same patient, by the same or different professionals in the same or different locations.

The [Eligible Professionals](#) section lists Part B providers who may provide diagnostic and therapeutic mental, psychoneurotic, personality disorder, and SBIRT treatment services allowed under state law.

We cover medically reasonable and necessary services or supplies to treat the patient's overall diagnosis and condition or improve a malformed body part. Services must meet standards of good medical practice for the diagnosis, direct care, and treatment of the patient's medical condition and must not be mainly for patient, provider, or supplier convenience.

Services must also meet specific [National Coverage Determination and Local Coverage Determination](#) medical necessity criteria.

Indicate the specific sign, symptom, or patient complaint for every service billed. Although a provider may consider a service or test good medical practice, we don't pay for services without patient symptoms, patient complaints, or specific documentation.

We also pay for multiple mental health services for the same patient on the same day. However, we don't pay for inappropriate or duplicate services on the same day. If you have questions about local or national policies that may prevent you from billing certain services, find your [Medicare Administrative Contractor's website](#).

[Substance Use Screenings & Treatment](#) and [SBIRT Services](#) have more information on SUD services.

Safety Planning Intervention & Follow-Up Contacts Intervention

SPI involves a patient working with a clinician to develop a personalized list of coping and response strategies and sources of support. Patients can use this list if they experience thoughts of harming themselves or others. This isn't a suicide risk assessment but an intervention for patients you determine have an elevated risk for suicide or risky substance use. You can use SPI to reduce the risk of overdose. The billing practitioner provides this service.

The basic components of a [safety plan](#) are:

- Recognizing warning signs of an impending suicidal crisis or actions that increase the risk of overdose
- Employing internal coping strategies
- Using social contacts and social settings as a means of distraction from suicidal thoughts or taking steps to reduce the risk of suicide or substance use
- Using family members, significant others, caregivers, and friends to help resolve the crisis
- Contacting mental health or SUD professionals, crisis services, or agencies
- Making the environment safe, including by restricting access to lethal means, as applicable

Note: Use HCPCS code G0560.

FCI is specific to patients with suicide risk, including those who have experienced an overdose. It involves a series of phone contacts, up to 4 calls per calendar month, between a provider and a patient in the weeks and sometimes months following any instance in which the patient has been discharged after a crisis encounter. This includes discharge from an ED or psychiatric inpatient care, or after crisis stabilization.

These calls happen when the person is in the community, and they're designed to reduce the risk of subsequent adverse outcomes. FCI calls are typically 10–20 minutes and:

- Encourage using the safety plan (as needed in a crisis) and updating it to optimize effectiveness
- Express psychosocial support
- Encourage follow-up care and services

Bill this service once per month, covering up to 4 calls. Because Part B cost-sharing applies, we require you to obtain verbal or written patient consent either before or during the first call. You may bill FCI incident to the services of the billing practitioner.

Note: Use HCPCS code G0544.

Prescription Drug Coverage

Medicare Parts A and B generally don't cover prescription drugs. However, Part B covers certain medications that patients can't self-administer, like drugs that must be given by a health care provider. For broader prescription drug coverage, patients must enroll in Medicare drug plans (Part D).

Part D must cover certain protected drug classes, including antipsychotics, antidepressants, and anticonvulsants. Drug plans must cover most medications in these drug classes, with some exceptions.

Medicare Advantage Organizations

Medicare Advantage (MA) enrollees can get Part A, Part B, and Part D benefits under a single plan. MA plans provide Part B-covered mental health services and may offer certain benefits (for example, telehealth services) beyond what Part B pays for. They may also provide supplemental benefits that Parts A and B don't cover (for example, supplemental mental health benefits may address coping with life changes, conflict resolution, or grief counseling).

Telehealth

Visit the [CMS Telehealth](#) webpage for the Medicare telehealth policy.

Starting January 1, 2026, for services without a 010 or 090 global surgery indicator, we permanently revised the definition of direct supervision to allow the immediate availability of the supervising practitioner through real-time audio-video interactive telecommunications, excluding audio-only.

Mental health services that Rural Health Clinics (RHCs) and Federally Qualified Health Centers (FQHCs) provide through telehealth are paid for under the All Inclusive Rate (AIR) and Prospective Payment System (PPS), respectively.

We created 3 Outpatient Prospective Payment System (OPPS)-specific HCPCS codes to describe that the patient must be in their home and that no associated professional service is billed under the Physician Fee Schedule (PFS). Hospital staff must be licensed to provide these services as is consistent with all applicable state scope-of-practice laws. We exempt these services from having staff physically located in the hospital or outpatient department when services are provided remotely using communication technology.

Table 1. Telehealth HCPCS Codes

Description	HCPCS Code
Service for diagnosis, evaluation, or treatment of a mental health or substance use disorder, 15–29 minutes, provided remotely by hospital staff who are licensed to provide mental health services under applicable state law(s), when the patient is in their home, and there is no associated professional service	C7900
Service for diagnosis, evaluation, or treatment of a mental health or substance use disorder, 30–60 minutes, provided remotely by hospital staff who are licensed to provide mental health services under applicable state law(s), when the patient is in their home, and there is no associated professional service	C7901
Service for diagnosis, evaluation, or treatment of a mental health or substance use disorder, each additional 15 minutes, provided remotely by hospital staff who are licensed to provide mental health services under applicable state law(s), when the patient is in their home, and there is no associated professional service (List separately in addition to code for primary service)	C7902

You can find more telehealth codes under [Commonly Used HCPCS/CPT Codes](#) as indicated by an asterisk (*). [Telehealth & Remote Monitoring](#) has more information.

Eligible Professionals

The following sections list the required qualifications and coverage requirements for these practitioners, as well as the PFS payment amounts they're eligible to bill and be paid under Part B:

- Physicians (MDs and DOs), particularly psychiatrists
- Clinical psychologists (CPs)
- Clinical social workers (CSWs)
- Clinical nurse specialists (CNSs)
- Nurse practitioners (NPs)
- Physician assistants (PAs)
- Certified nurse-midwives (CNMs)
- Independently practicing psychologists (IPPs)
- Certified registered nurse anesthetists (CRNAs) (supervision of diagnostic psychological and neuropsychological tests)
- Marriage and family therapists (MFTs)
- Mental health counselors (MHCs), including certified alcohol and drug counselors



Provider Information

These sections list individual provider-type required qualifications, coverage, and payment criteria. Each provider type must meet all qualifications and coverage requirements. See the [Commonly Used HCPCS/CPT Codes](#) section for specific billing codes.

Physician

Required Qualifications

- Be an MD or DO
- Be legally authorized to practice medicine in the state where you provide services
- Act within the scope of your license

Coverage Requirements

We cover reasonable and necessary services not precluded by law. Generally, in addition to performing tests, you may also supervise the performance of diagnostic psychological and neuropsychological tests.

You may have services and supplies provided incident to your personal professional services.

Payment

We pay at 100% under the [PFS](#).



Clinical Psychologist

Required Qualifications

- Have a psychology doctoral degree
- Be licensed or certified at the independent level in the state where you practice and directly provide diagnostic, assessment, preventive, and therapeutic patient services

Coverage Requirements

We cover reasonable and necessary services not precluded by law if you're legally authorized to practice psychology in the state where you provide services. Generally, in addition to personally performing diagnostic psychological and neuropsychological tests, you may supervise the performance of those tests.

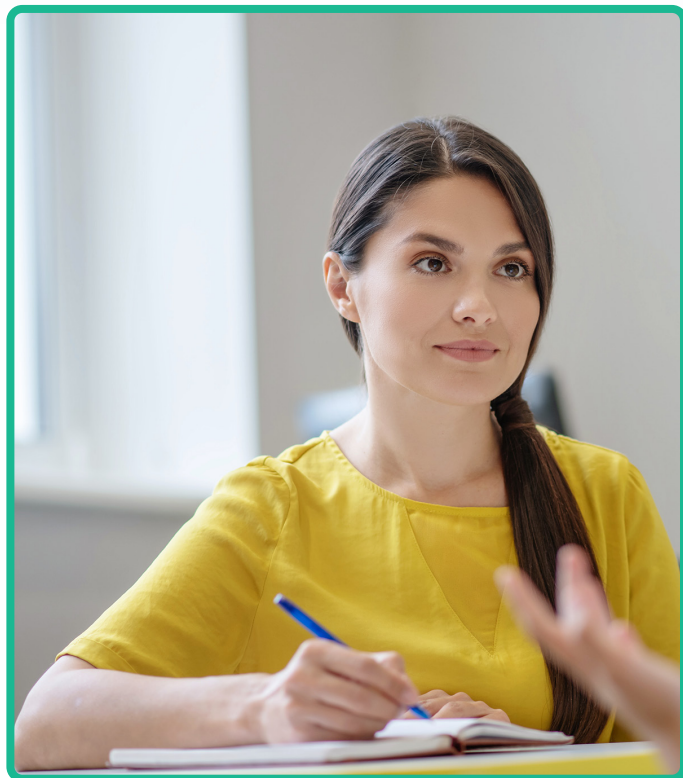
You may have services and supplies provided incident to your personal professional services.

Unless the patient's primary care or attending physician referred the patient, tell the patient that you'd like to consult their physician. Document in the medical record if the patient consented to or declined the consultation. If the patient consents to the consultation, document in their medical record either the:

- Consultation date
- Date you unsuccessfully tried to consult their physician and the method you used

Payment

We pay at 100% under the [PFS](#) only if you accept assignment.



Clinical Social Worker

Required Qualifications

- Have a social work master's or doctoral degree
- Have performed at least 2 years of supervised clinical social work
- Be licensed or certified in the state where you provide services
- Have at least 2 years or 3,000 hours of post-master's degree supervised clinical experience in social work practice in an appropriate setting (for example, a hospital, skilled nursing facility (SNF), or clinic) if you practice in a state that doesn't have licensure or certification

Coverage Requirements

We cover reasonable and necessary services performed to diagnose and treat a mental illness if they aren't precluded by law and if you're legally authorized to practice clinical social work in the state where you provide services.

We cover CSW hospital outpatient services and pay for CSW services under the CSW benefit category when hospitals bill under the CSW's NPI.

We don't pay you under the CSW benefit category for your:

- Hospital inpatient services
- Services to patients under a PHP or an IOP performed at a hospital outpatient department or CMHC
- Services to SNF inpatients and patients in Medicare-participating ESRD facilities if the services are under the respective provider's participation requirements

We may cover ancillary CSW services when you provide them as auxiliary personnel incident to the personal professional services of a physician, CP, CNS, NP, PA, or CNM.

We don't cover services provided incident to your personal professional services.

Payment

You must accept assignment. We pay at 75% of the [PFS](#) amount for CPs.

We pay Method II critical access hospitals (CAHs) 80% of the lesser of the actual charge for the service or 75% of the CP's PFS payment if you've reassigned your billing rights to the CAH.



Clinical Nurse Specialist

Required Qualifications

- Be a registered nurse (RN) currently licensed in the state where you practice and authorized to provide CNS services according to state law
- Have a Doctor of Nursing Practice or master's degree in a defined clinical nursing area from an accredited educational institution
- Be certified as a CNS by a recognized [national certifying body](#) with established CNS standards

Coverage Requirements

We cover reasonable and necessary services not precluded by law if you're legally authorized to practice medicine in the state where you provide services while working in collaboration with a physician. We cover services that we would consider physicians' services if an MD or a DO provided them.

We may cover assistant-at-surgery services you provide.

In collaboration with a physician, you may personally perform diagnostic psychological and neuropsychological tests to the extent authorized by state law. We authorize you to supervise the performance of diagnostic tests according to state law and scope of practice.

You may have services and supplies provided incident to your personal professional services.

Payment

You must accept assignment. You can't charge a patient more than the amounts permitted under [42 CFR 424.55](#). If a patient paid for a service over these limits, refund the excess amount.

We pay for professional services at 80% of the lesser of the actual charge or 85% of the amount a physician gets under the [PFS](#). We pay for your incident to services provided outside a hospital setting at 85% of the amount a physician gets under the PFS.

We pay for assistant-at-surgery services directly at 85% of the 16% that a physician gets under the PFS.



Nurse Practitioner

Required Qualifications

- Be an RN licensed and authorized, according to state law, by the state where you provide NP services
- Be a registered professional nurse who's authorized by the state where you provide services to practice as an NP by December 31, 2000
- Have gotten Medicare NP billing privileges for the first time:
 - On or after January 1, 2003, and:
 - Be NP-certified by a recognized [national certifying body](#) with established NP standards
 - Have a master's degree in nursing or a Doctor of Nursing Practice
 - Before January 1, 2003, and meet certification requirements
 - Before January 1, 2001



Coverage Requirements

We cover reasonable and necessary services not precluded by law if you're legally authorized to practice medicine in the state where you provide services while working in collaboration with a physician. We cover services we would consider physicians' services if an MD or a DO provided them.

We may cover assistant-at-surgery services you provide.

In collaboration with a physician, you may personally perform diagnostic psychological and neuropsychological tests to the extent authorized by state law. We authorize you to supervise the performance of diagnostic tests according to state law and scope of practice.

You may have services and supplies provided incident to your personal professional services.

Payment

You must accept assignment. You can't charge a patient more than the amounts permitted under [42 CFR 424.55](#). If a patient paid for a service over these limits, refund the excess amount.

We pay for professional services at 80% of the lesser of the actual charge or 85% of the amount a physician gets under the [PFS](#). We pay for incident to services that auxiliary personnel provide outside a hospital setting at 85% of the amount a physician gets under the PFS.

We pay for assistant-at-surgery services directly at 85% of the 16% that a physician gets under the PFS.

Physician Assistant

Required Qualifications

- Be licensed by the state where you practice
- Have graduated from a PA educational program accredited by the [Accreditation Review Commission on Education for the Physician Assistant](#) (or its predecessor agencies, the Commission on Accreditation of Allied Health Education Programs and the Committee on Allied Health Education and Accreditation)
- Have passed a national certification exam administered by the [National Commission on Certification of Physician Assistants](#)

Coverage Requirements

We cover reasonable and necessary services not precluded by law if you meet all PA qualifications and are legally authorized to practice medicine in the state where you provide services. Provide these services under a physician's supervision. We cover services we would consider physicians' services if an MD or a DO provided them.

We may cover assistant-at-surgery services you provide.

You may personally perform diagnostic psychological and neuropsychological tests under physician supervision as required under the PA benefit category and as authorized by state law. We authorize you to supervise the performance of diagnostic tests according to state law and scope of practice.

You may have services and supplies provided incident to your personal professional services.

Payment

You must accept assignment. You can't charge a patient more than the amounts permitted under [42 CFR 424.55](#). If a patient paid for a service over these limits, refund the excess amount.

Bill under your NPI. You can bill Medicare, and we pay for your services directly as we do NPs' and CNSs' services. You may reassign your service payment rights and incorporate as a group of only practitioners in your specialty and bill Medicare like NPs and CNSs.



We pay for:

- Professional services, including services and supplies provided incident to your services
- Professional services provided in all rural and non-rural settings and areas
- Services at 80% of the lesser of the actual charge or 85% of the amount a physician gets under the [PFS](#)
- Assistant-at-surgery services directly at 85% of the 16% that a physician gets under the PFS
- Services provided incident to a PA outside a hospital at 85% of the amount a physician gets under the PFS

We pay only if no facility or other provider charges or we didn't pay any other service amount they provided.

When you bill a hospital service (inpatient and outpatient) directly, we unbundle the payment and pay you directly.



Certified Nurse-Midwife

Required Qualifications

- Be an RN legally authorized to practice as a nurse-midwife in the state where you provide services
- Have successfully completed a nurse-midwives program of study and gotten clinical experience accredited by an accrediting body approved by the U.S. Department of Education
- Be certified as a nurse-midwife by the [American College of Nurse-Midwives](#)

Coverage Requirements

We cover reasonable and necessary services not precluded by law if you're legally authorized to practice medicine in the state where you provide services. We cover services we would consider physicians' services if an MD or a DO provided them.



Unless otherwise required by state law, we cover services you provide without physician supervision and without association with a physician or other health care provider.

You may personally perform diagnostic psychological and neuropsychological tests without physician supervision or oversight as required under the CNM benefit category and as authorized under state law. We authorize you to supervise diagnostic tests performed according to state law and scope of practice.

You may have services and supplies provided incident to your personal professional services.

Payment

You must accept assignment. You can't charge a patient more than the amounts permitted under [42 CFR 424.55](#). If a patient paid for a service over these limits, refund the excess amount.

We pay for services at 80% of the lesser of the actual charge or 100% of the amount a physician gets under the [PFS](#).

We pay incident to services provided outside a hospital at 100% of the amount a physician gets under the PFS.

Independently Practicing Psychologist

Required Qualifications

- Be a psychologist who isn't a CP
- Meet 1 of these criteria:
 - Practice independently of an institution, agency, or physician's office and be licensed or certified to practice psychology in the state or jurisdiction where you provide the services
 - Be a practicing psychologist who provides services in a jurisdiction that doesn't issue licenses



Coverage Requirements

We cover reasonable and necessary services not precluded by law.

We consider you practicing independently when you:

- Provide services on your own responsibility, free of administrative and professional control of an employer (for example, physician, institution, or agency)
- Treat your own patients
- Can bill directly and collect and retain service fees

When you practice in an office in an institution, we consider you an IPP when you meet both conditions:

- Your office is confined to a separately identified part of the facility that's used solely as your office and that can't be construed as extending throughout the entire institution
- You operate a private practice (serving patients from outside the institution as well as institutional patients)

You may perform diagnostic psychological and neuropsychological tests when physicians or certain NPPs order them.

Payment

We don't subject diagnostic psychological and neuropsychological tests to assignment. However, on the claim, include the name and address of the physician or NPP who orders the tests.

We pay for diagnostic tests at 100% under the [PFS](#).

Certified Registered Nurse Anesthetist

Required Qualifications

- Be licensed as a registered professional nurse by the state where you practice
- Meet any licensure requirements the state imposes on non-physician anesthetists
- Have graduated from a nurse anesthesia educational program that meets [Council on Accreditation of Nurse Anesthesia Educational Programs](#) (COA) standards or another accreditation organization the HHS secretary designates
- Within 24 months of graduation, have passed a certification exam from the [National Board of Certification & Recertification for Nurse Anesthetists](#) or another certification organization the HHS secretary designates

Coverage Requirements

We cover reasonable and necessary services not precluded by law if you're legally authorized to practice medicine in the state where you provide services.

You may personally perform diagnostic psychological and neuropsychological tests under physician supervision as required under the CRNA benefit category and as authorized by state law. We authorize you to supervise the performance of diagnostic tests according to state law and scope of practice.

You can bill directly and collect and retain service fees.

Payment

You must accept assignment. We pay at 100% under the [PFS](#) as determined by the level of required supervision.

We may pay you directly or pay another person or entity (for example, hospital, CAH, physician, group practice, or ambulatory surgical center) with which you have an employment or contractual arrangement.



Marriage & Family Therapist

Required Qualifications

- Have a master's or doctoral degree that qualifies you for licensure or certification as an MFT according to state law where you provide services
- Be licensed or certified as an MFT in the state where you provide services
- Have at least 2 years or 3,000 hours of post-master's degree clinical supervised experience in marriage and family therapy in an appropriate setting (for example, a hospital, SNF, or clinic)

Coverage Requirements

You must be legally authorized to practice as an MFT in the state where you provide services.

Since January 1, 2024, you may enroll in Medicare and bill Medicare independently.

We may cover ancillary MFT services when you provide them as auxiliary personnel incident to the personal professional services of a physician, CP, CNS, NP, PA, or CNM. We don't cover services provided incident to your personal professional services.

You can provide MFT services through an acceptable telehealth mental health disorder service site.

Payment

You must accept assignment. We pay 75% of the [PFS](#) amount for CPs.

We pay Method II CAHs 80% of the lesser of the actual charge or 75% of the amount a CP gets under the PFS if you've reassigned your billing rights to the CAH.

We don't pay under the MFT benefit category for MFT services a hospital outpatient department or CMHC provides to patients under a PHP or an IOP.

We exclude MFT services provided to SNF residents from [consolidated billing](#). Include the SNF's Medicare provider number when you bill for these Part B services.



Mental Health Counselor

Required Qualifications

- Have a master's or doctoral degree that qualifies you for licensure or certification as an MHC according to state law where you provide services.
- Be licensed or certified in the state where you provide services as 1 of these:
 - An MHC
 - A clinical professional counselor
 - An addiction, alcohol, or drug counselor
 - A professional counselor
- Have at least 2 years or 3,000 hours of post-master's degree clinical supervised experience in mental health counseling



Coverage Requirements

You must be legally authorized to practice as an MHC in the state where you provide services.

Since January 1, 2024, you may enroll in Medicare and bill Medicare independently.

We may cover ancillary MHC services when you provide them as auxiliary personnel incident to the personal professional services of a physician, CP, CNS, NP, PA, or CNM. We don't cover services provided incident to your personal professional services.

You can provide MHC services through an acceptable telehealth mental health disorder service site.

Addiction counselors or alcohol and drug counselors who meet the applicable MHC requirements can [enroll in Medicare as MHCs](#).

Payment

You must accept assignment. We pay 75% of the [PFS](#) amount for CPs.

We pay Method II CAHs 80% of the lesser of the actual charge or 75% of the amount a CP gets under the PFS if you've reassigned your billing rights to the CAH.

We don't pay under the MHC benefit category for MHC services a hospital outpatient department or CMHC provides to patients under a PHP or an IOP.

We exclude MHC services provided to SNF residents from [consolidated billing](#). Include the SNF's Medicare provider number when you bill for these Part B services.

“Incident To” Provision

Physicians and certain NPPs have a provision under their benefit category that authorizes them to have ancillary services and supplies provided by auxiliary personnel “[incident to](#)” their own personal professional services.

Physicians and specifically CPs, NPs, CNSs, CNMs, and PAs can bill and be paid for these integral, although incidental, services and supplies provided by auxiliary personnel as if they provided the services and supplies themselves if all the incident to requirements are met:

- Services and supplies are integral to the patient’s normal treatment course, and the physician or other listed NPP personally provided an initial service to which the auxiliary personnel’s services are incidental. The physician or NPP must remain actively involved in treating the patient.
- The auxiliary personnel provide services and supplies without charge (included in the physician’s or other listed NPP’s bill).
- Services and supplies are an expense to the physician or other listed NPP.
- Services and supplies are commonly provided in the physician’s or other listed NPP’s office or clinic.
- The physician or other listed NPP provides direct supervision and is available if needed.

Note: Under the PFS, we don’t pay physicians or NPPs for incident to services in an institutional setting (hospital or SNF).

We exempt behavioral health services provided by auxiliary personnel from the direct supervision requirement for incident to services. Incident to behavioral health services can be provided under the general supervision of a physician or an NPP. Under general supervision, the physician or NPP may be contacted by phone, if necessary, as the physician’s or NPP’s presence isn’t required during the service.

We don’t define behavioral health services by HCPCS codes; however, we generally understand a behavioral health service to be any service provided for the diagnosis, evaluation, or treatment of a mental health disorder, including an SUD.

Physicians, NPPs, and practitioners can also serve as auxiliary personnel and provide services and supplies incident to the personal professional services of another physician or NPP. Appropriate payment can be made to the other supervising physician, or NPP in this case, if you meet all the incident to requirements.

[42 CFR 410.26](#) and [42 CFR 410.27](#) have more information.

Outpatient Psychiatric Hospital Services

A hospital may provide a wide range of services and programs to its outpatients who need psychiatric care, ranging from a few individual services to comprehensive, full-day programs; and from intensive treatment programs to those that provide primarily supportive services.

Outpatient psychiatric hospital services and supplies are:

- Provided under an individualized, written plan of care (POC) (not required when you provide only a few brief services) that states the:
 - Type, amount, frequency, and duration of services
 - Diagnosis
 - Expected goals
- Supervised and periodically evaluated by a physician who:
 - Prescribes the services
 - Determines the extent to which the patient reached treatment goals and whether the POC should change
 - Provides supervision and direction to therapists treating the patient
 - Documents their involvement in the patient's medical record
- Intended for diagnostic study or reasonably expected to improve the patient's condition. At a minimum, the treatment is designed to reduce or control a patient's psychiatric symptoms to prevent a relapse or hospitalization and improve or maintain their level of functioning. See the [Same-Day Billing for Mental Health and Primary Care Services](#) section for more information.



Covered Outpatient Services

Generally, we cover these outpatient hospital (including PHP and IOP) psychiatric treatment services:

- Medically necessary diagnostic services for patients when extended or direct observation is necessary to determine functioning and interactions, identify problem areas, and prepare a POC.
- Individual and group psychotherapy with physicians, CPs, CSWs, or other eligible providers authorized or licensed by the state where they provide services (for example, MFTs and certified alcohol and drug counselors).
- Occupational therapy services, when part of a PHP or an IOP, that:
 - Require qualified occupational therapist skills
 - Are provided by, or under supervision of, a qualified occupational therapist
 - Are included in a patient's POC
- Services of social workers, psychiatric nurses, and other staff trained to work with psychiatric patients, including patients with SUD.
- Activity therapies, when part of a PHP or an IOP, that:
 - Are individualized and essential for treating a patient's diagnosed condition and progress toward treatment goals
 - Have a POC that clearly supports and shows each therapy's need (not primarily recreational or diversionary)
- Family counseling services whose primary purpose is treating a patient's condition. These services include counseling services for caregivers.
- Patient training and education when they're closely and clearly related to care and treating a patient's diagnosed psychiatric condition. These services include caregiver training services provided for the patient's benefit.
- Therapeutic drugs and biologicals a patient can't self-administer.
- Other items and services that:
 - Are reasonable and necessary for diagnosing or actively treating the patient's condition
 - Can be reasonably expected to improve or maintain the patient's condition and functional level
 - Help prevent relapse or hospitalization

Occupational Therapy POC Signature Requirements

Starting January 1, 2025, the physician's or NPP's signature on the order or referral counts as a signature on the POC if that physician or NPP hasn't signed and returned the patient's POC to the occupational therapist within 30 days of the initial evaluation. This applies only when the patient's physician or NPP signs and dates the written order or referral and indicates the type of therapy needed. Include that written order or referral in the patient's medical record.

Non-Covered Outpatient Services

Generally, we don't cover these outpatient hospital (including PHP and IOP) services:

- Meals and transportation
- Activity therapies, group activities, or other primarily recreational or diversionary services and programs
- Outpatient psychosocial programs (we cover outpatient psychosocial components not primarily for social or recreational purposes)
- Vocational training related only to specific employment opportunities

Partial Hospitalization Program

[PHPs](#) are distinct and structured programs that provide intensive outpatient psychiatric care, including SUD services, through active treatment by combining clinically recognized items and services. We cover PHPs in hospital outpatient departments and CMHCs.

PHPs offer psychiatric treatment less than 24 hours a day to patients:

- Discharged from an inpatient hospital treatment program, with the PHP replacing continued inpatient treatment
- At reasonable inpatient hospitalization risk without partial hospitalization



Patients may pay a percentage of each doctor's or other qualified mental health professional's approved service amount if the provider accepts assignment. Patients may also pay each day's [coinsurance](#) for PHP services in a hospital outpatient setting or CMHC.

When a hospital provides non-partial hospitalization mental health services to a partial hospitalization patient, report all partial hospitalization services and non-partial hospitalization mental health services on the same hospital claim with condition code 41.

We require non-excepted off-campus provider-based departments of a hospital to report modifier PN on each claim line for non-excepted items and services. Excepted off-campus provider-based departments of a hospital should report modifier PO for all excepted items and services provided.

PHPs must meet certain [conditions and exclusions](#) and program and patient criteria.

Program Criteria

- Active treatment includes an individual POC with coordinated services designed for the patient's needs and includes a physician-directed multidisciplinary team care approach
- Treatment goals should be:
 - Measurable
 - Functional
 - Time-framed
 - Medically necessary
 - Directly related to the admission reason

Patient Criteria

- Be under the care of a physician who [certifies](#) the patient's need for partial hospitalization therapeutic services a minimum of 20 hours per week and recertifies this need no less frequently than every 30 days
- Require a comprehensive, highly structured, scheduled, and multi-modal individualized POC requiring medical supervision and coordination because the patient's mental disorder or SUD severely interferes with multiple areas of daily life (including social, vocational, or educational functioning)
- Be able to cognitively and emotionally participate in the active treatment process and tolerate its intensity

Intensive Outpatient Program

Intensive outpatient services are provided under an [IOP](#).

Outpatients may get IOP services from their hospital, or through a CAH, a CMHC, an FQHC, an RHC, or an OTP as a distinct and organized intensive ambulatory treatment service, offering less than 24-hour daily care, in a location other than a person's home or inpatient or residential setting.

Physicians prescribe an IOP for a person determined (not less frequently than once every other month) to need these services for a minimum of 9 hours per week. They're provided under the physician's supervision pursuant to an individualized, written treatment plan or therapy POC established and periodically reviewed by the physician (in consultation with appropriate staff participating in such a program). This states the physician's diagnosis; the type, amount, frequency, and duration of the items and services provided under the plan; and the treatment goals.



Intensive outpatient services must include a physician certification and POC. However, although a PHP requires the physician to certify that the services are in place of inpatient hospitalization, IOP services aren't intended for those who otherwise need an inpatient level of care.

In addition to physicians, these NPPs may perform the required certification and the POC requirements for IOP services provided in the OTP setting:

- NPs
- PAs
- CPs
- CSWs
- MHCs
- MFTs
- Any other NPPs defined in section 1842(b)(18)(C) of the [Social Security Act](#), as permitted by state law and consistent with scope-of-practice requirements

For IOP services, a hospital's non-excepted off-campus provider-based departments are required to report modifier PN on each claim line for non-excepted items and services. A hospital's excepted off-campus provider-based departments must continue to report modifier PO for all excepted items and services provided.

When a hospital provides non-intensive outpatient program mental health services to an IOP patient, all intensive outpatient services and non-intensive outpatient mental health services should be reported on the same hospital claim with condition code 92 in form locators 18–28 to indicate the claim is for IOP services.

Note: Certain IOP services aren't payable as RHC or FQHC services. For example, group therapy is considered an IOP service and payable as part of the IOP payment amount but wouldn't be payable if billed as an RHC or FQHC service.



Community Mental Health Centers

We cover Part B partial hospitalization services that CMHCs provide, subject to the OPPS. Medicare-authorized CMHCs must meet these [program and patient criteria](#):

- Have appropriate state and local CMHC licensing or certification
- Provide:
 - Outpatient services, including specialized services for children, older adults, chronically mentally ill patients, and residents of the CMHC's service area discharged from an inpatient mental health treatment facility
 - 24-hour emergency care services with clinician access and appropriate disposition and with follow-up documentation of the emergency in the patient's CMHC medical record
 - Day treatment, partial hospitalization services, intensive outpatient services, or psychosocial rehabilitation services with structured daily treatment plans varying in intensity, frequency, and duration according to the patient's needs
 - At least 40% of the CMHC's services to patients who are ineligible for [Social Security Act, Title XVIII](#) benefits
 - Clinically evaluated state mental health facility candidate admissions performed by clinical personnel and authorized under state law, except those provided by a 24-hour facility; a CMHC operating in a state that, by law, prevents it from providing these services may contract with an entity the HHS secretary approves

A CMHC is an originating [telehealth services](#) site.

Behavioral Health Integration Services

Integrating behavioral health and primary care helps improve patient mental and behavioral health condition outcomes. We separately pay physicians and NPPs providing BHI services over a calendar month.

CPs, CSWs, MFTs, and MHCs can bill the general BHI HCPCS code G0323 when they're personally performing services to account for monthly care integration and when those services are the focal point of care integration. We allow general supervision for G0323.

[Behavioral Health Integration Services](#) has more information.

Opioid Treatment Programs

We pay certified opioid treatment programs (OTPs) through bundled opioid use disorder (OUD) Part B treatment services payments based on weekly episodes of care. We cover treatment for OUD for as long as reasonable and necessary.

Covered items and services include:

- FDA-approved opioid agonist and antagonist MOUD:
 - Methadone, including initiating treatment with methadone provided through 2-way interactive audio-video technology
 - Buprenorphine, including extended-release buprenorphine injection (under the brand name Brixadi®)
 - Naltrexone
 - Nalmefene hydrochloride nasal spray (under the brand name Opvee®)
- Dispensing and administering MOUD, if applicable
- Substance use counseling
- Individual and group therapy
- Toxicology testing
- Intake activities
- Periodic assessments
- Take-home supplies of methadone, oral buprenorphine, naloxone, and nalmefene
- Intensive outpatient program services
- Coordinated care and referral services
- Patient navigational services
- Peer recovery support services

[Opioid Treatment Program Directory](#) and [OTP Billing & Payment](#) have more information, including HCPCS codes and billing for OTPs.

Audio-Video Telehealth

You can provide these services through 2-way audio-video interaction:

- Substance use counseling and individual and group therapy services included in the bundled payment
- The add-on code for additional counseling and therapy
- Intake activities and periodic assessments
- Initiation of treatment with buprenorphine

Note: Provided the OTP meets all other applicable requirements, you can also provide these services through **audio-only** interaction (for example, phone calls) when audio-video communication isn't available to the patient. This includes circumstances where the patient isn't capable of, or doesn't consent to, using devices that permit a 2-way audio-video interaction. Audio-only communication can be used for both new and established patients.

HCPCS Code G2076

OTPs can use the intake add-on HCPCS billing code G2076 when they start methadone treatment with audio-only devices. We allow this if the patient is with a licensed practitioner who can prescribe and dispense controlled medications and if audio-video technology isn't available or practical. This licensed practitioner must be in the same room as the patient to do the visual part of the exam.

Inpatient Psychiatric Facility Services

Inpatient psychiatric facilities (IPFs) include freestanding certified psychiatric hospitals and psychiatric units in acute care hospitals or CAHs, providing routine hospital and psychiatric services to diagnose and treat patients' mental disorders.

We pay for inpatient psychiatric services under the [IPF Prospective Payment System](#) (PPS) when the facility is certified and meets [inpatient psychiatric hospital services regulations](#).

We require updated hospital inpatient rights and [discharge planning conditions of participation](#) for these hospital types:

- Short-term acute care
- Rehabilitation
- Psychiatric
- Children's
- Cancer
- CAHs

IPF Medical Records Requirements

IPF medical records must show the physician or NPP treatment level and intensity for each patient they admit to the hospital, among other requirements detailed at [42 CFR 482.61](#).

Patients must be able to access their medical records when requested verbally or in writing, and the hospital must quickly meet the patient's request, as detailed at [42 CFR 482.13](#).

Inpatient Hospital Services

When a physician admits a patient to the hospital for inpatient psychiatric facility services, we cover the services only if the patient needs intensive, appropriate, and active treatment in this type of setting. The psychiatric facility must be a general hospital with a distinct psychiatric unit or a psychiatric hospital that cares only for people with mental health conditions.

We certify IPFs and distinct psychiatric units in acute care hospitals and CAHs.

We cover:

- Semi-private rooms
- Meals
- General nursing
- Drugs (including methadone to treat OUD)
- Other inpatient hospital treatment services and supplies

[Deductible and coinsurance](#) apply. See the IPF Coverage Period section for more information.

If appropriate, physicians can admit patients to a general acute care hospital that doesn't have a distinct psychiatric unit to get mental health and SUD services. These inpatient services are covered like other [inpatient services in a general acute care hospital](#).

IPF Coverage Period

We cover IPF patient services in specialty facilities for 90 days per illness with a 60-day lifetime reserve and 190 days of care in freestanding psychiatric hospitals (this 190-day limit doesn't apply to certified psychiatric units).

The patient gets no more benefits after using 190 days of psychiatric hospital care.

Under the IPF PPS, federal per diem rates include inpatient operating and capital-related costs (including routine and ancillary services). We base them on:

- Geographic factors
- Patient characteristics
- Facility characteristics

IPFs get additional payments for:

- Patients treated in IPFs with a qualifying ED
- The number of ECT treatments provided
- Outlier cases (cases with extraordinarily high costs)

[Medicare Benefit Policy Manual, Chapter 2](#) has more information on how we cover IPFs.



Commonly Used HCPCS/CPT Codes

Using the correct HCPCS or CPT code to show the mental health services you provide to patients is essential for billing correctly.

Note: Only certain codes are billable as part of a PHP or an IOP. View the [Medicare Claims Processing Manual, Chapter 4](#), sections 260 and 261 for more information.

Table 2. Commonly Used Mental Health-Related HCPCS/CPT Codes

Code Category	HCPCS/CPT Code
Cognitive assessment and care planning*	99483
Interactive complexity*	90785
Psychiatric diagnostic evaluation*	90791, 90792
Psychotherapy with patient*	90832, 90833, 90834, 90836, 90837, 90838
Psychotherapy for crisis*†	90839, 90840
Psychotherapy for crisis in an applicable site of service†	G0017, G0018
Psychoanalysis*	90845
Family psychotherapy*	90846, 90847
Multiple-family group psychotherapy*	90849
Group psychotherapy (other than of a multiple-family group)*	90853
Electroconvulsive therapy (ECT)	90870
Hypnotherapy	90880
Preparation of report of patient's psychiatric status, history, treatment, or progress	90889
Neurobehavioral status exam*	96116, 96121
Psychological testing evaluation services*	96130, 96131
Neuropsychological testing evaluation services*	96132, 96133
Psychological or neuropsychological test administration*	96136, 96137, 96138, 96139
Psychological or neuropsychological test administration, with automated result	96146
Health behavior assessment or re-assessment*†	96156
Health behavior intervention*†	96158, 96159, 96164, 96165, 96167, 96168, 96170, 96171

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Table 2. Commonly Used Mental Health-Related HCPCS/CPT Codes (cont.)

Code Category	HCPCS/CPT Code
Administration of caregiver-focused health risk assessment instrument*	96161
Multiple-family group behavior management/modification training*	96202, 96203
Behavior identification assessment*	97151
Behavior identification-supporting assessment*	97152
Adaptive behavior treatment by protocol*	97153, 97154
Adaptive behavior treatment with protocol modification*	97155, 97158
Adaptive behavior treatment guidance*	97156, 97157
Caregiver training*	97550, 97551, 97552, G0539, G0540, G0541, G0542, G0543
Post discharge telephonic follow-up contacts performed in conjunction with a discharge from the emergency department for behavioral health or other crisis encounter, 4 calls per calendar month	G0544
Interprofessional telephone/internet/electronic health record assessment and management service	G0546, G0547, G0548, G0549, G0550
Interprofessional telephone/internet/electronic health record referral services	G0551
Supply of DMHT device and initial education and onboarding	G0552
Monthly treatment management of patient's therapeutic use of DMHT device	G0553, G0554
Administration of a standardized, evidence-based assessment of physical activity and nutrition*†	G0136
Safety planning interventions*†	G0560
Initiation of medication for the treatment of opioid use disorder in the emergency department setting	G2213

* Current telehealth codes.

† CPs, CSWs, MFTs, and MHCs can bill these codes.

HCPCS code G0136 can also be provided with CPT code 90791 (Psychiatric diagnostic evaluation) and the HBAI services, described by CPT codes 96156, 96158, 96159, 96164, 96165, 96167, and 96168. We allow CPs, CSWs, MFTs, and MHCs to bill the HBAI services described by those CPT codes and any successor codes.

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National Correct Coding Initiative

The [National Correct Coding Initiative](#) (NCCI) promotes national correct coding methods and offers national guidance on code pair edits preventing the billing of certain services on the same day for the same patient.

Resources

- [Addressing & Improving Behavioral Health](#)
- [Medicare Benefit Policy Manual, Chapters 2, 6, and 15](#)
- [Medicare Claims Processing Manual, Chapters 3 and 4](#)
- [Notices and Forms](#)
- [Opioid Treatment Programs](#)
- [Quality Improvement Organizations](#)
- [SAMHSA: Mental Health](#)

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